



REVIEW DRAFT • v2 • NOT YET LIVE

The Hospital Sentence Bank

All 35 sentences — revised, new, and questionable ones tagged. Nothing dropped.

Three expert panels • mom-first filter • Shelbi Kohler, certified birth doula

How to read this

 **REVISED**

Changed after panel review — old wording shown inside the card.

 **FLAGGED**

A panel raised a concern. Kept in — your call on each.

 **NEW**

Covers the gaps panels found: holding a no (3.5–3.7), intuition (5.5–5.6).

CLEAN

Unchanged, unchallenged.

Rule applied to every change: the mom stays peaceful, productive, in control. Nothing softened for the hospital. 11 revised · 5 new · 5 flagged · 14 untouched.



1 • Induction Pressure

5 sentences for this moment.

△ FLAGGED

1.1

“Can you walk me through why you're recommending induction today, and what my alternatives are?”

When your provider suggests starting labor before it starts on its own — at a checkup, over the phone, or at the hospital.

Why it asks for the *reason* and the *options* in one calm breath. It tells your team you're paying attention without challenging their judgment, and it buys you a real conversation instead of a yes.

Next you'll get the medical picture (your baby's status, your body, your timeline) and usually a range of choices — wait and monitor, schedule it, or go home and come back. You and your provider decide together.

△ **The flag:** Questionable — panel split on length: 17 words reads desk-length mid-contraction. Proposed short version: “Why today — and what are my other options?” Your call: full, short, or both printed on the card.

CLEAN

1.2

“Thank you — let's talk through the timing. What happens if we wait two more hours? Two more days?”

When the recommendation feels urgent, but nobody has explained what “urgent” means for you specifically.

Why urgency is real sometimes, and vague sometimes. Asking for the timeline in concrete terms turns “we need to do this now” into information you can actually weigh. It keeps you collaborative while slowing the moment down.

Next you'll hear what monitoring looks like while you wait, and what signs would change the plan. Most of the time you'll leave with a decision you helped make.

🔧 REVISED

1.3

“What makes this the right call for my situation specifically — and what would change that?”

was: “This sounds like the standard protocol. Can we talk about what makes sense for my situation specifically?”

When the recommendation arrives with urgency but without context, and you want it applied to *you*, not the average patient.

Why it asks for the personalization directly — the same firm ask without the “standard protocol” opener that puts a provider on defense before the real question lands. It tells your team you're paying attention, and it invites them to think with you instead of at you.

Next your provider walks through your specifics. Sometimes the answer is still the protocol — and now you'll know why it's right for you, which is a completely different feeling.

🔧 REVISED

1.4

“If we choose to start induction, what will the first steps look like, and how long is each one?”

When you've decided to move forward and you want the picture before anything starts.

Why agreeing and asking for the roadmap keeps you in the driver's seat even when you've said yes. You'll know what to expect step by step, with rough timing, which lowers the fear of the unknown — the loudest thing in most labor rooms.

Next your nurse or provider maps the sequence (medication, monitoring, checks). You know what each step is for before it happens.

“We need a few minutes alone to talk this through — please come back in ten minutes.”

was: “We'd like a few minutes alone to talk about it before we decide. Is now a good time to have that space?”

When the room fills up with people and opinions and you haven't had a second to hear yourself think.

Why it's clear, calm, and completely reasonable — and it's your right. Stating it plainly, with a return time, keeps the decision in your hands: you're not asking permission to think, and the team knows exactly when to come back.

Next the team steps out, you and your partner (or you and your doula) talk, and you come back to the conversation ready. That pause is often where the real decision lives.



2 • Epidural & Pain Relief Discussion

4 sentences for this moment.

“I'd like to understand my pain options — including trying without medication for now. Can we go over them all?”

was: “I'd like to understand my pain options — including doing nothing medically for now. Can we go over them all?”

When early in labor, or when pain relief comes up before you've thought it through.

Why it puts *all* the options on the table — including waiting — without signaling a decision either way. It makes pain management a conversation, not a checkbox.

Next you'll hear the full menu (epidural, other medications, breathing and movement tools, or a mix). Nothing is decided until you decide.

“What does getting an epidural involve right now — and what would happen if I waited a bit longer?”

When the conversation is moving toward an epidural and you want the moment to slow down before you commit.

Why it treats the epidural as a real choice with timing, not an automatic next step. Asking “what would happen if I wait” is a completely reasonable question that keeps your options open.

Next you'll learn how the epidural works, how long setup takes, and what waiting does or doesn't change. Then you choose — with the facts, either way.

“I want to try a while longer without the epidural — and I might change my mind. Is that okay?”

was: “I'd like to try a while longer without it, and I want you to know that could change. Can we agree that's okay?”

When you want to keep going unmedicated for now but you don't want anyone keeping score.

Why it states your current choice AND pre-permits yourself to change your mind. Nobody has to guess where you stand, and both options stay open without judgment.

Next your team knows the plan and knows it's flexible. You labor on your terms, and the door stays open both ways.

“Before we decide on the epidural — what will it change, and what won't it change?”

was: “The epidural is on the table and we're considering it. Before we decide, can you tell us what it will and won't change?”

When you're genuinely torn and want the honest trade-offs, not a sales pitch in either direction.

Why it asks for truth, not persuasion. A good care team will tell you what it changes (pain relief, mobility), what it may affect (length of pushing, need for oxytocin), and what it does not: the epidural does not increase your chance of a cesarean. You decide from reality.

Next you hear the real picture and make the call that fits *your* labor. Both answers are fine; only one is yours.



3 • Unexpected Intervention Recommendation

7 sentences for this moment.

CLEAN

3.1

“That’s new information for us. Can you explain what’s changed and what the options are now?”

When something shifts in labor (monitoring changes, your contraction pattern changes, or new information comes in) and the plan you prepared for suddenly isn’t the plan anymore.

Why it acknowledges the moment (“new information”), asks for the reason, and asks for options — without panic and without refusal. It’s the sentence that keeps you a participant when everything speeds up.

Next your team explains the change. You get the updated picture and the updated choices. The plan changes; your role in deciding doesn’t.

CLEAN

3.2

“What are the risks if we do this, and what are the risks if we wait and monitor?”

When a recommendation arrives mid-labor and you need to weigh it quickly.

Why it frames *both sides* of the decision — doing something and doing nothing both have trade-offs. Asking about both makes you an informed decision-maker in under thirty seconds.

Next you’ll hear both pictures. Sometimes waiting is clearly right; sometimes acting is. Either way, the decision is now yours, made from both sides.

REVISED

3.3

“I hear you, and I know this is serious. Can we have a minute to take this in before we decide?”

was: “I hear you, and I trust this is serious. Can we have a moment to absorb this and then decide together?”

When the moment is heavy — a recommendation you didn’t expect, delivered quickly.

Why it does two things at once: accepts the seriousness (your team needs to know you’re not ignoring them) and claims the space to think. Serious moments still deserve a breath. If your team says we have to act now — an emergency — let them. This sentence is for the serious-but-stable moments.

Next the team gives you the moment (they almost always do when asked directly), then returns for your decision. You’ll make it present, not panicked.

REVISED

3.4

“Is there a smaller step we can try first, before we do that?”

was: “Is there a version of this we can try first that’s less of a change from our current plan?”

When you want to meet the concern the team raised without jumping to the biggest step.

Why it asks for intermediate options — the step between “nothing” and “everything.” It keeps your preferences in the decision instead of replacing them. Sometimes the honest answer is no — hearing no quickly is also good information.

Next your team either offers a middle path (great — take it and stay watched) or explains why there isn’t one. Both answers are useful information.

+ NEW

3.5

“I understand, and I’m declining this for now — what are my other options?”

When you’ve heard the team out and your answer is no — for now.

Why voluntary decline is the core of consent. This holds your no calmly while keeping the conversation open — you’re declining this step, not the relationship.

Next your team either offers alternatives (yours to accept or decline) or explains the urgency. To put the no on record: “Can you note in my chart that I’m declining for now?”

+ NEW

3.6

“Is that a recommendation or an ultimatum? I want to make this decision myself.”

When the recommendation starts sounding like a directive and the pressure in the room rises.

Why it names the moment calmly and puts the decision back where it belongs — with you. Most providers will immediately reset to a real recommendation.

Next you’ll usually hear the honest version: what’s recommended, what’s optional, and what’s genuinely urgent.

+ NEW

3.7

“What happens if we don't do this today?”

When you want to understand the real stakes of waiting before you decide.

Why it turns vague pressure into concrete information — and reminds the room that waiting is a real option with real trade-offs.

Next you'll hear what waiting means in your situation. Sometimes it's reassuring; sometimes it's a genuine reason to act. Either way, the decision is yours, made with both sides known.



4 • Consent & Paperwork

4 sentences for this moment.

CLEAN

4.1

“Before I sign, can you explain what I'm agreeing to in plain language?”

When a clipboard appears with a form and a quick “just sign here.”

Why “plain language” is the magic phrase — it tells them you want to understand, not recite. Consent you actually understood is consent that means something.

Next they walk you through it in real words. If something doesn't make sense, ask again — you haven't held anyone up; you've done your job.

CLEAN

4.2

“What happens if I don't sign this right now?”

When the pace feels fast and the signature feels automatic.

Why it's a completely fair question asked without accusation. The answer tells you whether this is truly urgent or just routine — and those are very different situations.

Next you'll hear which it is. Most of the time there's more room than the pen-in-hand moment suggests, and you sign when you know what you're signing.

CLEAN

4.3

“Can I have a copy of this to keep, and a few minutes to read it?”

When any consent form, honestly — but especially ones that arrived mid-labor.

Why reading your own paperwork is not an inconvenience; it's your right. Asking for a copy also signals you take it seriously — which makes the whole exchange more respectful.

Next you get the form and the time. You read, you ask, you sign when you're ready.

CLEAN

4.4

“Can someone go through the specific risks listed here with me?”

When the form has a wall of fine print and nobody's explained it line by line.

Why it turns fine print into a conversation and puts a real person in charge of explaining it. Nobody should consent to a list of words they can't explain.

Next a nurse or provider sits with you and translates the list. You sign knowing what each item means for you.



5 • Communication Breakdowns

6 sentences for this moment.

REVISED

5.1

“I think we're talking about different things. Let me try again — what I'm asking is: ____.”

was: “I think we're talking past each other. Can we back up — here's what I understand, and here's what I'm asking.”

When you've asked something twice and gotten an answer to a different question.

Why it names the breakdown without blame and rebuilds in one breath — one blank to fill, not a speech to deliver. Handing them the exact sentence makes it easy to correct precisely the part that's off.

Next the misunderstanding gets located and fixed, usually in one exchange. The conversation moves forward with both of you in it.

CLEAN

5.2

“I'm hearing conflicting information. Can you help me understand how this fits together?”

When your nurse says one thing, the doctor says another, and you're caught in the middle trying to make sense of it.

Why it puts the *team* back in “care team” — you're not choosing sides or reporting anyone, you're asking them to reconcile their own picture. It's respectful and it works.

Next someone takes ownership of the full picture and explains how the pieces fit. Sometimes this genuinely improves your care, not just your understanding.

REVISED

5.3

“This matters a lot to me — I need an answer before we move on. Can we take it now?”

was: “This matters a lot to me and I want to make sure it doesn't get missed. Can we come back to it?”

When you've said something that matters and it's sliding off the conversation.

Why it names the stakes and asks for the answer now, not later — direct without being adversarial, hard to ignore and hard to be offended by.

Next most providers reset and truly engage. You get your point back into the conversation, where it belongs.

CLEAN

5.4

“Could you write down the key points, or can my support person take notes while we talk?”

When the information is coming fast and you know you won't remember it accurately.

Why it normalizes documentation in the room. Notes protect *you* (memory, decisions) and *them* (clear understanding), so both sides usually welcome it.

Next the details land on paper. Afterward you can reread, and your support person can help you process what was said.

+ NEW

5.5

“Give me a minute — I need to check in with myself before I answer.”

When you're asked to decide and your gut hasn't caught up with the conversation.

Why the I in BRAIN — intuition — deserves a voice. This claims the moment to hear yours, without explaining or apologizing.

Next you get the minute. Ask your body what it says, then answer from both places — gut and head.

+ NEW

5.6

“Something feels off to me, and I need you to take that seriously.”

When you sense something is wrong, but you don't have data to point to.

Why this is a sentence doulas teach word for word. A mom's instinct is information — this says so out loud, in terms a team can act on.

Next a good team will re-check the monitors and re-examine. If they wave it off, repeat it and add: “Can you check and tell me what you see?”



6 • Scheduling & Pace Pressure

4 sentences for this moment.

△ FLAGGED

6.1

“I understand there's a schedule to keep. What's the medical reason for that timing in my case?”

When things are moving on the ward's rhythm — and you want to know how much of it is medicine and how much is logistics.

Why it separates medicine from logistics politely. Hospitals have real schedules; asking which reasons are medical puts the focus back where it belongs.

Next you'll get the medical picture — and the honest parts about logistics. Then you decide with clear eyes about what's driving what.

△ **The flag:** Questionable — OB panel called it presumptuous; discarded per mom-first rule. Mom + doula panels say keep verbatim. Included so you see it was raised.

🔍 REVISED

6.2

“We're not ready yet. If it's safe to wait a bit longer, could we have a little more time — and how much?”

was: “We're not ready yet, and we're comfortable with where things are. Can we have more time?”

When you're being asked to move to a next step and your gut says not yet.

Why it's simple and firm — states where you are, checks that waiting is safe, and asks for the time in concrete terms. “Not ready yet” is a legitimate answer, and the safety check keeps the wait collaborative instead of a standoff.

Next usually, you get time. Sometimes you'll hear a reason why waiting matters — and that's information worth having, offered in the same calm register.

CLEAN

6.3

“If we wait another hour, what would we be watching for that would change our minds?”

When you want to buy time *and* stay safe — the smart combination.

Why it turns waiting from passive into active. You're telling the team: we'll wait, and we want to know the specific signs that would make waiting the wrong call. That's what collaboration looks like.

Next the team defines the watch-points with you. Time passes with everyone watching the same things — which is exactly how waiting should work.

△ FLAGGED

6.4

“Can we plan for the rest of today together, so we're not deciding each step under pressure?”

When the day has become a chain of small decisions, each one arriving faster than the last.

Why it lifts the conversation from step-by-step pressure to a shared map. A patient who wants to plan the day is a patient the team can actually work with — and a shared map beats a chain of pressured micro-decisions.

Next you walk the day's likely path together, including the decision points. When each step arrives, it's familiar instead of sudden.

△ **The flag:** Sentence kept; the “providers generally prefer this too” claim is being cut from the why (overpromise).



7 • Being Alone or Unsupported in the Room

5 sentences for this moment.

CLEAN

7.1

“I’d like my support person (my doula/partner) in the room for this conversation. When can they come back in?”

When a shift change, a procedure, or a hallway conversation is happening and your person got left outside.

Why it frames your support person as part of the team — because they are. Asking “when can they come back” treats their absence as temporary logistics, not a policy.

Next your person rejoins you, usually quickly. Support in the room changes every conversation that follows.

CLEAN

7.2

“I’m feeling alone in this and I need some support right now. Can someone stay with me?”

When the room is quiet, everyone’s at the desk, and you’re doing the hardest work of your life by yourself.

Why asking directly for support is strength, not weakness — and it gives the team something specific and doable. Nurses want to know this. Most will stay, or send someone who can.

Next someone stays with you, checks in, or explains when they’ll be back. You’re not doing this alone anymore.

REVISED

7.3

“Before any procedure starts, can you make sure my support person is here with me?”

When something is about to happen and you want your people beside you for it.

Why it makes your support person’s presence a standing part of the plan rather than something you renegotiate each time. One sentence, stated once. If it’s a true emergency, the team will act first and bring your person in after — and some procedures, like a cesarean, have their own limits on how many people can be present.

Next the team builds the wait-for-my-person into the routine. You go into every procedure with your people.

FLAGGED

7.4

“Who will be in the room for the birth itself, and can we talk about who I want there?”

When the room’s population is growing and you haven’t been asked.

Why it asks for the guest list in advance — a completely normal request that’s easy to forget you’re allowed to make. Your birth, your room.

Next you find out who’s planning to be there and you get a real say. Most teams honor this fully once it’s voiced.

△ **The flag:** Mild flag — fine as written; guest limits genuinely vary by unit, the why already hedges.

FLAGGED

7.5

“Can you stay while we talk this through? I do better when I can ask questions in the moment.”

When someone important is about to leave the room right when you need them.

Why it gives the provider a reason to stay that costs nothing. “Questions in the moment” is how good medical conversations work — you’re just asking for the good version.

Next they stay, you talk, and the decision happens in one complete conversation instead of fragments.

△ **The flag:** Mild flag — fine as written, checked only against the one-breath standard.

Disclaimer — one change

was: “...always follow their recommendations for medical decisions.”

“True Joy Birthing provides birth education, not medical advice. These sentences are tools for informed conversations with your care team — medical decisions are made together with your providers.”

Your call

- 1.** 11 revised sentences
- 2.** 5 new cards
- 3.** disclaimer rewrite
- 4.** rulings on the 5 flagged
- 5.** then the IG visual pass

Nothing changes on the site until you say go. — Kit